

**BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES
REDRESSAL COMMISSION, GHAZIABAD (U.P.)**

CONSUMER COMPLAINT NO. _____ OF 2026

(Complaint under Section 35 of the Consumer Protection Act, 2019)

IN THE MATTER OF:

Mr. Vikas Gandhi

S/o Shri Surendra Gandhi, aged about 53 years,

R/o D-20, 1st Floor, Dayanand Nagar,

Ghaziabad, Uttar Pradesh – 201001

Mobile: 9911198000 | Email: 9amdeal@gmail.com

...COMPLAINANT

VERSUS

ACKO General Insurance Limited

Through its Managing Director / Authorised Signatory

Regd. Office: 2nd Floor, 36/5 Hustle Hub One East,

Somasandrapalya, 27th Main Road, Sector 2, HSR Layout,

Bengaluru, Karnataka – 560102

Email: gro@acko.com | IRDAI Reg. No. 157

...OPPOSITE PARTY

**COMPLAINT UNDER SECTION 35 OF THE CONSUMER PROTECTION
ACT, 2019 FOR ARBITRARY REPUDIATION OF A MEDICALLY
NECESSARY HEALTH CLAIM, DEFICIENCY IN SERVICE AND UNFAIR
TRADE PRACTICE**

MOST RESPECTFULLY SHOWETH:

The present complaint arises out of the repudiation of a fully documented hospitalization claim of ₹37,358/- despite **uncontroverted medical evidence** — the hospital record of acute urinary obstruction with active infection, an urgent surgery performed by a super-specialist Urologist, a biopsy confirming a premalignant disease, and the treating specialist's written certification — none of which the Opposite Party has ever examined, answered or rebutted. Every material assertion in this complaint is supported by a document annexed hereto.

CHAPTER I — PARTIES

1. The Complainant is a "consumer" within the meaning of Section 2(7) of the Consumer Protection Act, 2019, having purchased a health insurance policy from the Opposite Party for consideration for personal/family use.
2. The Opposite Party is a general insurance company registered with IRDAI (Reg. No. 157), rendering insurance services for consideration.

CHAPTER II — JURISDICTION AND LIMITATION

3. **Territorial:** The Complainant resides at Ghaziabad and the entire hospitalization and treatment took place at RS Maitri Health Care, J-278, Patel Nagar 1st, Ghaziabad. This Hon'ble Commission has territorial jurisdiction under Section 34(2)(d) of the Act.
4. **Pecuniary:** The claim and compensation sought are well within the pecuniary limits of the District Commission.
5. **Limitation:** The claim was repudiated on 26.06.2025; the Complainant thereafter pursued the insurer's grievance mechanism and the Insurance Ombudsman, whose Award was pronounced on 02.07.2026. The complaint is within two years of the cause of action (Section 69). The Ombudsman's own forwarding letter dated 02.07.2026 (Annex-XIII-A) records, verbatim:

"In the eventuality of your disagreeing with the enclosed award, you may, move a fresh application at any other forum/court that may be considered by you as appropriate."

CHAPTER III — CHRONOLOGY OF EVENTS

Date	Event	Supporting Document
04.11.2024	Proposal accepted after underwriting; ACKO Personal Health Policy (Standard Plan, Family Floater) No. ASHP202400001607 issued; Sum Insured ₹10,00,000; premium ₹31,065 p.a.; policy period 04.11.2024–03.11.2025	Annexure C-1 (Policy Schedule); Annexure C-2 (Proposal Form)
Nov 2024 – Jun 2025	All premium instalments paid on time; policy continuously in force without break	Annexure C-3 (payment records)
June 2025	Complainant develops acute urinary complaints — poor flow of urine with infection	Annexure C-4 (Discharge Summary)
20.06.2025	Admitted to RS Maitri Health Care, Ghaziabad at 06:58 hrs; Circumcision + Meatoplasty performed the same day under spinal anaesthesia by Dr. Vaibhav Saxena, M.Ch. (Urology); IV fluids and IV antibiotics administered; excised tissue sent for histopathology	Annexure C-4
21.06.2025	Discharged in stable condition; total hospital expenses ₹37,358/-	Annexure C-4; Annexure C-5 (Bills)
21/24.06.2025	Reimbursement Claim No. 25062100350/1 lodged with all documents; detailed representation sent	Annexure C-6
25.06.2025	Claim rejected by "Update on Your Claim" email quoting the genitourinary entry and asserting — five days before the histopathology report existed — that the procedure "is not related to a malignancy"; GRO separately rejects the representation in identical mechanical terms	Annexure C-7
26.06.2025	Claim repudiated by email citing only the general "Specific Illnesses" waiting-period clause, without naming any listed condition and without dealing with the exception contained in it	Annexure C-8
30.06.2025	Histopathology Report (Max Lab, No. V-6630/25): gross "cauliflower like lesion 3.5 × 3 × 0.8 cm"; Impression: "Consistent with Lichen sclerosus (balanitis xerotica obliterans)"	Annexure C-9
Jul–Aug 2025	Appeals and grievances to the Opposite Party — all rejected without dealing with the medical evidence	Annexure C-10 (colly.)

07.08.2025	Complaint filed before Insurance Ombudsman, Noida (NOI-H-056-2526-0961)	Annexure C-11
24.06.2026	Personal hearing before the Ombudsman	Annexure C-11
02.07.2026	Ombudsman Award IO/NOI/A/HI/0061/2026-2027: complaint dismissed, but medical necessity held undisputed and Permanent Exclusion held inapplicable ; liberty to approach any other forum	Annexure C-11; Annex-XIII-A letter
12.07.2026	Treating Urologist issues Medical Certificate: surgery "was must" due to inability to pass urine and UTI; biopsy confirmed BXO, "a pre-malignant condition"; risk of life-threatening infection and penile cancer if not treated	Annexure C-12
14.07.2026	Grievance registered on the National Consumer Helpline (INGRAM) portal against the repudiation — Grievance No. 9719631 (status: in process)	Annexure C-10 (colly.)
15.07.2026	Opposite Party's Grievance Redressal Officer replies by email, reiterating verbatim the very "waiting period" ground of 26.06.2025 — once again without identifying any listed condition in the Schedule, and without adverting to the biopsy report, the treating doctor's certificate dated 12.07.2026, or the Ombudsman's findings in paras 13–14 of the Award	Annexure C-10 (colly.)

CHAPTER IV — THE MEDICAL FACTS: AN ACUTE ILLNESS AND A MEDICALLY UNAVOIDABLE SURGERY

6. In June 2025 — more than seven months into the policy — the Complainant developed **acute obstruction to the passage of urine**. The Discharge Summary of RS Maitri Health Care (Annexure C-4) records, verbatim:

Diagnosis: "TIGHT PHIMOSIS WITH BALANITIS WITH MEATAL STENOSIS"

Brief history of presenting illness: "POOR FLOW OF URINE"

Procedure: "CIRCUMCISION + MEATOPLASTY DONE UNDER SA ON 20/06/2025"

Course during hospitalization: "IV FLUIDS AND IV ANTIBIOTICS"

7. These four entries of the hospital's own contemporaneous record establish, without any need for argument: (i) acute urinary obstruction; (ii) **active infection** — balanitis with urinary tract infection (UTI), treated with intravenous antibiotics, as also certified by the treating Urologist (Annexure C-12); (iii) surgery performed **on the very day of admission**, which is possible only where surgery is medically unavoidable; and (iv) treatment by a qualified surgical team under anaesthesia in a hospital setting.

8. The excised preputial tissue was sent for histopathological examination — a step which was **clinically indicated**, because the Histopathology Report of Max Lab dated 30.06.2025, No. V-6630/25 (Annexure C-9), records in its Gross Description, verbatim:

"Externally shows cauliflower like lesion measures 3.5 x 3 x 0.8 cm."

A "cauliflower-like" growth is a morphology that any competent surgeon is duty-bound to have examined for malignancy.

9. The Report's Impression reads, verbatim:

"Consistent with Lichen sclerosus (balanitis xerotica obliterans)."

The biopsy thus **confirmed BXO (Balanitis Xerotica Obliterans / Lichen Sclerosus)** — a recognised premalignant condition of the penis, as certified by the treating specialist (Chapter V below) and as recorded in standard medical literature (Annexure C-13).

10. The medical sequence is therefore complete and entirely documentary: acute urinary obstruction → recorded poor urinary flow → active infection requiring IV antibiotics → medically unavoidable urgent surgery → clinically indicated biopsy → biopsy-confirmed BXO. Not one link in this chain rests on the Complainant's say-so; every link is a hospital or laboratory record.

CHAPTER V — OPINION OF THE TREATING SUPER-SPECIALIST

11. The treating surgeon and certifying specialist is **Dr. Vaibhav Saxena, M.S. (BHU), M.Ch. Urology (AIIMS, New Delhi)**, U.P. Medical Council Reg. No. 47425, who, as per his professional letterhead and registration stamp, is:

- **Senior Director – Urology, Uro-Oncology (Cancer), Robotic Surgery and Renal Transplant**, Yashoda Super Speciality Hospital, Kaushambi;
- Consultant Urologist, RS Maitri Health Care, J-278, Patel Nagar 1st, Ghaziabad; and
- **the treating surgeon himself**, who operated upon the Complainant and followed up the histopathology.

12. This is not the opinion of an ordinary physician rendered on paper review. It is the certification of a **super-specialist in urological cancer who personally examined, operated upon and followed up the patient**. His Medical Certificate dated 12.07.2026 (Annexure C-12), issued "To Whom It May Concern", certifies verbatim:

"Mr. Vikas Gandhi, 54 yrs Male, R/o D-20, Dayanand Nagar, Ghaziabad was suffering from Phimosis with Balanitis with meatal stenosis & UTI. His surgery was must in view of unable to pass urine & UTI. His Preputial Biopsy came out to be (BXO) which is a pre-malignant condition & not any elective or cosmetic surgery. There is life threatening infection risk & risk of penile cancer if not treated..."

The certificate thus establishes, in the treating specialist's own hand, that:

- (a) the patient suffered from phimosis with balanitis, meatal stenosis **and UTI**;
- (b) the surgery **"was must"** — medically unavoidable — in view of inability to pass urine and UTI;
- (c) the preputial biopsy confirmed **BXO, "a pre-malignant condition"**;
- (d) the surgery was **not elective or cosmetic**; and
- (e) without treatment there was a risk of **life-threatening infection** and of **penile cancer**.

13. It is respectfully submitted that the opinion of the treating super-specialist carries the **highest evidentiary value** in a medical-claim dispute. The Opposite Party has never produced — at the claim stage, the grievance stage, or before the Ombudsman — **any**

contrary opinion of any doctor whatsoever, let alone of an equally qualified uro-oncologist. An insurer cannot arbitrarily discard the unrebutted opinion of the treating specialist on the basis of a desk assessment by unnamed, unqualified claim processors.

CHAPTER VI — THE PROPOSAL FORM: NO SUPPRESSION, NO PRE-EXISTING DISEASE

14. The Proposal Form (Annexure C-2) — the Opposite Party's own document, on the basis of which it underwrote and accepted the risk — records, verbatim:

"Has anyone in this plan been diagnosed with a medical condition?" — **"No medical conditions"**

"Did anyone in this plan take any prescribed medicines in the past week?" — **"No"**

"Has anyone in this plan been advised to undergo or has ever undergone hospitalisation for any illness / surgery?" — **"no"**

"Does anyone in this plan have any other ongoing health issues or symptoms for which you are planning a doctor visit?" — **"No"**

"PED declared" — **"Obesity"**

15. Thus: the **only** condition declared was obesity; there was no previous medical condition, no ongoing symptom, no previous surgery and no previous hospitalization. The Opposite Party accepted this proposal **after underwriting** and issued the policy recording, in its own Schedule (Annexure C-1), **"Permanent disease exclusion: No exclusion."**

16. It is of decisive significance that the Opposite Party has **never** alleged suppression of material facts, has **never** invoked the Pre-Existing Disease clause, and has **never** questioned the genuineness of the illness or of the treatment. Any suggestion that the disease existed prior to policy inception therefore stands completely demolished by the insurer's own record and its own conduct.

16-A. Indeed, the Opposite Party has itself **renewed** the policy as Policy No. ASHP202400001607R1 for the period 04.11.2025–03.11.2026 (Sum Insured ₹11,00,000 including inflation-protection bonus; premium ₹36,656.86 — Annexure C-1 colly.). The renewal document records **"First policy inception date: 04/11/2024"** and "Existing user:

Yes". The insurer thus continues to accept the Complainant's premium and treats the cover as continuous from 04.11.2024 — even while refusing the only claim ever made under it.

CHAPTER VII — THE REPUDIATION: MECHANICAL, NON-SPEAKING AND ARBITRARY

17. The entire repudiation email dated 26.06.2025 (Annexure C-8), insofar as it gives reasons, states verbatim:

"After carefully reviewing your claim, we noticed that you have filed a claim for the expenses incurred for treatment of a condition falling under waiting period. Unfortunately, these costs are not a part of your insurance coverage. As a result, we are unable to approve your claim."

followed only by an extract of the general clause 4.1.2 ("Specific Illnesses"). A day earlier, by its "Update on Your Claim" email dated 25.06.2025 (Annexure C-7), the Opposite Party had cited the genitourinary entry and declared that *"this procedure is not related to a malignancy"* — a **medical conclusion pronounced five days before the histopathology report (30.06.2025) even existed**, and without reference to any medical opinion whatsoever.

18. This Hon'ble Commission may test the quality of this decision against the record:

Question	Answer on the record
Did ACKO have any medical basis for its assertion of 25.06.2025 that the procedure "is not related to a malignancy"?	No — the histopathology report came into existence only on 30.06.2025, five days after that assertion.
Did ACKO discuss the biopsy report ?	No.
Did ACKO discuss BXO or its premalignant character?	No.
Did ACKO discuss the treating Urologist's opinion ?	No.
Did ACKO analyse the exception contained in the very clause it relied on ("unless necessitated by malignancy")?	No.
Did ACKO obtain any independent medical opinion ?	No.
Did ACKO refer the matter to any Medical Board ?	No.
Did ACKO give any scientific reason why a biopsy-confirmed premalignant condition cannot fall within the exception?	No — not even before the Ombudsman.

19. A repudiation that ignores every piece of material medical evidence, cites a clause without applying it to the facts, and pronounces a medical conclusion ("not related to a

malignancy") **before the histopathology report even existed**, is a **mechanical, non-speaking and arbitrary** decision. Quoting the clause is not application of mind; applying it to the medical evidence is — and that never happened at any level.

CHAPTER VIII — THE OMBUDSMAN PROCEEDINGS HAVE NARROWED THE CONTROVERSY

20. The Insurance Ombudsman, Noida, by Award No. IO/NOI/A/HI/0061/2026-2027 dated 02.07.2026 (Annexure C-11), dismissed the complaint; however, the Award records findings which bind the controversy and are no longer open to the Opposite Party, verbatim:

Para 13: *"It is noted that it is not disputed that the procedure was medically necessary and it was undertaken to address the complaint/problem of poor flow of urine, and it has also been conveyed by complainant that he had serious infection."*

Para 14: *"In this sense, the procedure involving circumcision + meatoplasty would not be covered under the Permanent Exclusion clause 4.2.3 of the policy as it has been necessitated by illness and forming part of treatment."*

21. Consequently, before this Hon'ble Commission the controversy is a **narrow** one: whether a surgery performed for a biopsy-confirmed premalignant condition falls within the exception *"unless necessitated by malignancy"*. Medical necessity, genuineness of treatment, active policy, and inapplicability of the Permanent Exclusion all stand concluded.

22. The Ombudsman decided the narrow point against the Complainant only because, at the hearing, there was *"no post operative medical paper"* certifying the malignant potential of BXO. That gap in the record no longer exists: the treating super-specialist's certificate (Annexure C-12) and the supporting medical literature (Annexure C-13) are now on record. The Award, rendered in the summary procedure of the Insurance Ombudsman Rules 2017, does not bar this complaint; its own covering letter grants liberty to approach any other forum.

CHAPTER IX — THE POLICY CLAUSE, READ LAST AND READ FAIRLY, SUPPORTS THE CLAIM

23. The Policy Schedule (Annexure C-1) prescribes for the Complainant, verbatim: *"Specific illness — 2 years waiting period on specific illness — NOTE: Refer Annexure for the complete disease list"*. The only entry of that Annexure relied upon by the Opposite Party reads, verbatim:

"Any Surgery of the genitourinary system *unless necessitated by malignancy*."

24. The clause is therefore **not an absolute exclusion**: the insurer itself carved out genitourinary surgery connected with malignancy from the waiting period. On the admitted and documented facts — a grossly suspicious "cauliflower like lesion", a biopsy-confirmed premalignant disease, and the treating uro-oncologist's certification that BXO is *"a pre-malignant condition"* carrying, if untreated, the *"risk of penile cancer"* — the present surgery was **"necessitated by malignancy"** within the true object and purpose of that exception.

25. It is significant what the policy **nowhere** says. The policy:

- nowhere **defines** "necessitated by malignancy";
- nowhere **excludes premalignant conditions** from the exception;
- nowhere requires an **existing or established cancer**;
- nowhere requires **histologically proven invasive carcinoma**; and
- nowhere states that **preventive oncological surgery** is excluded.

The insurer's reading — that the exception operates only after cancer has fully established itself — is therefore not the only possible interpretation, and it is an interpretation that defeats the evident object of the exception: that genuinely oncological genitourinary surgery must not be made to wait out two years, because in such cases delay is medically dangerous. A construction that pays for surgery only after the cancer arrives, but refuses the very surgery that prevents it, frustrates both the clause and the purpose of health insurance.

26. To be precise: the Complainant does **not** contend that he had established cancer. The contention — certified by the treating uro-oncologist and supported by standard medical literature — is that **BXO is a medically recognised premalignant condition carrying a known risk of progression to penile squamous cell carcinoma**, and that the surgery was

undertaken because of that documented malignant potential, not for any cosmetic or elective reason.

27. **In the alternative, and only if necessary:** if this Hon'ble Commission finds the expression "necessitated by malignancy" to be genuinely capable of two reasonable meanings — (i) surgery for established malignancy, or (ii) surgery necessitated by a disease process of malignant potential — then, by the settled rule of construction of insurance contracts (contra proferentem), the ambiguity in a clause drafted solely by the insurer must be resolved **in favour of the insured**. This submission is made strictly in the alternative; the Complainant's primary case rests on the medical evidence itself.

CHAPTER X — DEFICIENCY IN SERVICE AND UNFAIR TRADE PRACTICE

28. Repudiating a genuine, medically necessary, fully documented claim — without examining the biopsy, without obtaining any medical opinion, without applying the exception in the insurer's own clause, and without disclosing even the sub-clause relied upon — constitutes **deficiency in service** within Section 2(11) and **unfair trade practice** within Section 2(47) of the Consumer Protection Act, 2019. The arbitrary repudiation has caused the Complainant financial loss, prolonged harassment across three fora over more than a year, and mental agony.

CHAPTER XI — CAUSE OF ACTION

29. The cause of action arose on 20–21.06.2025 (hospitalization), 26.06.2025 (repudiation), 25.06.2025 and thereafter (grievance rejections), and finally on 02.07.2026 (Ombudsman Award with liberty to approach any other forum); it is continuing and arose within the jurisdiction of this Hon'ble Commission.

CHAPTER XII — PRAYER

30. It is therefore most respectfully prayed that this Hon'ble Commission be pleased to:

(a) **direct the Opposite Party to pay ₹37,358/-** (Rupees Thirty-Seven Thousand Three Hundred Fifty-Eight only), the admissible claim amount under Claim No. 25062100350/1;

(b) **award interest** thereon @ 12% p.a. (or such rate as deemed fit) from 26.06.2025 (date of repudiation) till realisation;

(c) **award compensation of ₹1,00,000/-** (or such sum as deemed fit) for mental agony and harassment caused by the arbitrary repudiation and protracted proceedings;

(d) **award litigation costs of ₹25,000/-**; and

(e) pass such other and further orders as this Hon'ble Commission may deem fit and proper.

AND FOR THIS ACT OF KINDNESS, THE COMPLAINANT AS IN DUTY BOUND SHALL EVER PRAY.

Place: Ghaziabad

Date: ____ July 2026

COMPLAINANT

(Vikas Gandhi)

VERIFICATION

I, Vikas Gandhi, the Complainant above-named, do hereby verify that the contents of this complaint are true and correct to my knowledge and belief, derived from the documents annexed hereto, and nothing material has been concealed therefrom.

Verified at Ghaziabad on this ____ day of July, 2026.

COMPLAINANT

LIST OF ANNEXURES

Annexure	Document
C-1 (colly.)	Policy Schedule, Policy No. ASHP202400001607 (with "Additional conditions" and Annexure – Specific illness list); renewal Policy Schedule No. ASHP202400001607R1 (04.11.2025–03.11.2026)
C-2	Proposal Form (recording "No medical conditions"; "PED declared — Obesity")
C-3	Premium payment records / policy active status
C-4	Discharge Summary, RS Maitri Health Care,

	Ghaziabad (20–21.06.2025)
C-5	Hospital bills with break-up of charges (₹37,358/-)
C-6	Claim registration and Complainant's representation dated 24.06.2025
C-7	GRO's rejection email dated 25.06.2025
C-8	Repudiation email dated 26.06.2025
C-9	Histopathology Report, Max Lab, dated 30.06.2025 (No. V-6630/25)
C-10 (colly.)	Subsequent appeals/grievance correspondence with the Opposite Party
C-11	Award of Insurance Ombudsman, Noida, No. IO/NOI/A/HI/0061/2026-2027 dated 02.07.2026, with forwarding letter (Annex-XIII-A)
C-12	Medical Certificate dated 12.07.2026 of the treating super-specialist, Dr. Vaibhav Saxena, M.S. (BHU), M.Ch. Urology (AIIMS, New Delhi), Senior Director – Urology, Uro-Oncology, Robotic Surgery & Renal Transplant, Yashoda Super Speciality Hospital, Kaushambi; U.P. Medical Council Reg. No. 47425
C-13 (colly.)	Medical literature on BXO / Lichen Sclerosus as a premalignant condition: (C-13A) EAU Guidelines on Penile Cancer 2022 (highlighted pages); (C-13B) Bunker CB, Shim TN, “Male Genital Lichen Sclerosus”, Indian J Dermatol (review) — see note below
C-14	Copy of Aadhaar / address proof of Complainant

Note on Annexure C-13 (Medical Literature — on record, colly.)

Two authoritative medical documents are annexed as C-13 (colly.), establishing that BXO / genital Lichen Sclerosus is a recognised premalignant condition associated with penile squamous cell carcinoma, and that phimosis (the Complainant's recorded diagnosis) is itself strongly associated with invasive penile cancer:

1. C-13A — European Association of Urology (EAU) Guidelines on Penile Cancer (2022):

Table 1 records phimosis as a risk factor with an odds ratio of 11–16 versus no phimosis and states that “Phimosis is strongly associated with invasive penile cancer ... due to associated chronic infection”; and Table 4 expressly classifies “Lichen sclerosis” among “Premalignant penile lesions (precursor lesions)” — the very biopsy diagnosis in the present case.

2. C-13B — Bunker CB, Shim TN, “Male Genital Lichen Sclerosus” (Review Article, Indian Journal of Dermatology):

records that the feared complication of male genital lichen sclerosis is squamous cell carcinoma of the penis; that “the association of GLSc and SCC is widely recognized and the risk has been estimated at between 2% and 12.5%” (bracketing the “about

6%” figure noted at the Ombudsman hearing); and that lichen sclerosus is one of the two recognised pathways for the development of penile cancer. This review also cites both Nasca et al. (J Am Acad Dermatol 1999) and Depasquale et al. (BJU Int 2000), the primary sources on the point.

(Relevant highlighted pages are annexed; the treating specialist may also be requested to reference/append this literature to his certificate at Annexure C-12.)

Working draft prepared from the documentary record for review by the Complainant's advocate before filing. An affidavit in support, complaint fee and paper-book sets are to be prepared per the Commission's filing requirements.